



Alberta Public Health Disease Management Guidelines

Hepatitis A



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Health and Wellness Promotion Branch

Public Health and Compliance Branch

Alberta Health

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Case Definition

Confirmed Case

- Acute clinical illness^(A) with laboratory confirmation of infection:
 - Positive hepatitis A virus immunoglobulin M antibody (anti-HAV IgM) in the absence of recent immunization^(B) with hepatitis A vaccine

OR

- Detection of HAV RNA (e.g., PCR) from an appropriate clinical specimen (e.g. stool, blood)

OR

- Positive anti-HAV IgM with or without clinical illness^(A):

- with an epidemiological link to a confirmed case,

AND

- absence of recent immunization^(B) with hepatitis A vaccine

Probable Case

- Acute clinical illness^(A) in a person who is epidemiologically linked to a confirmed case and is also *without laboratory confirmation of infection*^(C)

OR

- Positive anti-HAV IgM in the absence of:

- clinical illness^(A)

OR

- recent immunization^(B) with a hepatitis A-containing vaccine

^(A) Acute clinical illness is characterized by discrete onset of symptoms usually including fever, malaise, anorexia, nausea and abdominal pain followed by jaundice. Elevated serum aminotransferase levels accompanied by any of these symptoms is indicative of disease. Children may have non-specific or no symptoms. Asymptomatic infections with positive anti-HAV IgM should be confirmed by PCR to see if viremia is present.

^(B) Anti-HAV IgM has been detected up to two to three weeks after one dose of hepatitis A vaccine.⁽¹⁾

^(C) *Without laboratory confirmation of infection* means **no** specimens were collected for testing.

Reporting Requirements

Physicians, Health Practitioners and Others

Physicians, health practitioners and others shall notify the Medical Officer of Health (MOH) (or designate) of the zone, of all confirmed and probable cases in the prescribed form by the Fastest Means Possible (FMP).

Laboratories

All laboratories shall report all positive laboratory results:

- by FMP to the MOH (or designate) of the zone, and
- by mail, fax or electronic transfer within 48 hours (two business days) to the Chief Medical Officer of Health (CMOH) (or designate).

Alberta Health Services and First Nations Inuit Health Branch

Cases in Sensitive Situations or Occupations (SSO), or Where Media Exposure is Anticipated

- The MOH (or designate) of the zone where the case currently resides shall notify the CMOH (or designate) **by FMP** of all confirmed and probable cases.
- The MOH (or designate) of the zone where the case currently resides shall forward the initial Notifiable Disease Report (NDR) of all confirmed and probable FMP cases to the CMOH (or designate) within **one week** of notification and the final NDR (amendment) within **two weeks** of notification.

All Other Alberta Cases

- The MOH (or designate) of the zone where the case currently resides shall forward the initial NDR of all confirmed and probable cases to the CMOH (or designate) within **two weeks** of notification and the final NDR (amendment) within **four weeks** of notification.

Out-of-Province/Country Cases and Contacts

- For out-of-province and out-of-country reports, the following information should be forwarded to the CMOH (or designate) by FMP:
 - name,
 - date of birth,
 - out-of-province health care number,
 - out-of-province address and phone number,
 - positive laboratory report, and
 - other relevant clinical/epidemiological information.
- For out-of-province and out-of-country contacts who may be eligible for post-exposure prophylaxis (PEP), the following information should be forwarded to the CMOH (or designate) by FMP:
 - name,
 - date of birth, and
 - out-of-province address and phone number.

Epidemiology

Etiology

Hepatitis A virus (HAV) is an RNA virus within the *Hepatovirus* genus of the *Picornaviridae* family.^(1,2) There are six genotypes, three of which infect humans: genotypes I, II and III⁽³⁾ with further subdivision into subtypes A and B within these three genotypes, hence, IA, IB, IIA, IIB, IIIA and IIIB. However, HAV has a single conserved antigenic neutralization site and therefore these three genotypes also constitute a single serotype.⁽⁴⁾

Clinical Presentation

Hepatitis A (HA) infection is an acute, self-limited infection of the liver that can vary from asymptomatic to severe disease lasting a few months.^(1,5) The likelihood of developing symptomatic illness from HA infection is directly related to age, with only 30% of infected children younger than six years of age showing symptoms; if illness does occur, few of these children will have jaundice.^(5–7)

For those with symptoms, a person typically presents with non-specific mild, flu-like symptoms (referred to as prodromal phase).^(1,5,8) This is followed by onset of dark urine and jaundice (icteric phase), which generally begins within 7–10 days of the onset of the initial symptoms.^(3,8,9) The icteric phase lasts from 4–30 days.⁽¹⁰⁾ In general, severity increases with age; however, most cases completely recover without sequelae or recurrence of illness.⁽¹⁾

Prolonged or relapsing disease lasting 6–12 months occurs in approximately 15% of cases.^(1,5) Chronic HA infection is not known to occur. Fulminant hepatitis is rare but can occur more frequently in individuals with underlying liver disease or immunocompromising conditions.⁽⁵⁾ The reported case fatality rate among reported cases of all ages is low (~0.3%), but can be higher among persons over 50 or who have liver disease (~2%).⁽¹⁾

Diagnosis

Hepatitis A is difficult to distinguish clinically from other forms of viral hepatitis. Laboratory diagnosis is made by the demonstration of IgM antibody to HAV (anti-HAV IgM) in the serum of acutely or recently ill persons.^(1,3,5,6) Anti-HAV IgM usually becomes detectable 5–10 days before the onset of symptoms and may remain detectable for up to six months. Rarely, HAV IgM can remain detectable years after an acute infection. IgG appears in the convalescent phase of infection and persists for life, conferring lifelong immunity.

False positive anti-HAV IgM results can occur in individuals who have no evidence of recent infection (e.g., clinical illness) or exposures (low positive predictive test value), or due to cross-reaction with other serum factors or medications.^(3,11,12)

Indeterminate anti-HAV IgM results can be confirmed by PCR at Public Health Laboratory (ProvLab) by special request. PCR is not done routinely on initial screening. Acute/recent infection should be confirmed with clinical history, symptoms and/or by repeat titre after 7–10 days (if indicated) or by PCR (if required) after consultation with the virologist-on-call at ProvLab.

Samples from locally-acquired cases or clusters may also be submitted via the ProvLab to the National Microbiology Laboratory for genotyping.

Treatment

There is no cure for HA infection and measures are usually supportive only.

Reservoir

Humans.^(1,2)

Transmission

HA infection is primarily spread via fecal-oral person-to-person contact or less frequently by ingesting fecally-contaminated food or water.^(1,2,5) Household or other close contact is the most commonly reported source of HA infection.⁽²⁾ The secondary attack rate ranges between 20–50%.⁽¹³⁾

Although more rare, HA infection can be acquired through sexual contact (anal-oral), vertically (intrauterine transmission during the first trimester) and blood transfusions.^(2,3,14,15) In the case of transmission by blood transfusion, the donor must be in the viremic prodromal phase of infection at the time of blood donation.⁽⁶⁾ HAV has been detected in saliva in animals in the laboratory setting and therefore may be present in human saliva, but there is no evidence of this as a route of transmission.^(3,16,17)

HAV is partially resistant to heat. It is relatively stable at moderate temperatures but can be inactivated by high temperatures (85°C or higher), formalin or chlorine.⁽²⁾

Incubation Period

The incubation period of HA infection can vary between 15 and 50 days, depending on the infectious dose, with an average of 28 to 30 days.⁽¹⁾

Period of Communicability

Maximum communicability of HAV occurs during the latter half of the incubation period, two weeks before the onset of jaundice, and declines a few days after onset of jaundice.^(1,2,5,6) For anicteric (no jaundice) cases, this period is during peak aminotransferase activity. Most cases are not considered infectious after the first week of jaundice. However, prolonged viral excretion (up to six months) has been documented in infants and children ≤ five years of age.⁽¹⁾

In HIV positive individuals, the duration of viremia is significantly longer, which could imply a longer period of shedding in the feces and hence communicability.⁽¹⁸⁾ Chronic shedding of HAV does not occur, although relapses can occur, resulting in recurrent shedding.^(1,6)

For asymptomatic cases, the period of communicability can be estimated based on the timing of contact with the source, if known (e.g., contact with an index case or consumption of contaminated food), and when the laboratory test results occurred.⁽¹⁹⁾

Host Susceptibility

Individuals at increased risk for acquiring infection include:

- recent immigrants or travellers to hepatitis A-endemic countries,
- household/close contacts of children adopted from hepatitis A-endemic countries,
- individuals who work with non-human primates,
- men who have sex with men (MSM), and
- injection and non-injection drug users (IDU and non-IDU).⁽²⁰⁾

Individuals at risk of developing severe complications of hepatitis A include:

- those with chronic liver disease,
- hepatitis B carriers,
- anti-HCV positive individuals, and
- those who have had or will be undergoing liver transplantation.

Incidence

HA occurs worldwide and is notifiable nationally.⁽²¹⁾ Between 2010 and 2018, an average of 38 cases were reported each year in Alberta.⁽²²⁾ The majority of these (> 53%) were reported in individuals who had travelled to, or recently emigrated from, a developing country.

Common source outbreaks have been linked to:

- consumption of contaminated ice/water,
- ingestion of food contaminated by infected food handlers (including uncooked foods or foods handled after cooking),
- eating raw or undercooked shellfish harvested from contaminated waters, or
- fruits, vegetables and other foods that were contaminated during harvesting or subsequent handling and eaten uncooked.^(23–28)

Refer to the [Interactive Health Data Application \(IHDA\)](#) for the incidence of hepatitis A in Alberta.

Public Health Management

Key Investigation

- Confirm that the individual meets the case definition.
- Obtain a history of illness including the date of onset, signs and symptoms. For the purpose of public health follow-up, date of onset is the first day of prodromal phase **OR** the seventh day prior to the onset of jaundice, if prodrome is not known (See [Clinical Presentation](#) for more information).
 - Determine the dates of communicability (period of infectiousness).
 - Identify any underlying medical conditions that may increase host susceptibility.
- Determine the occupation of the case (e.g., food handler, childcare facility worker, health care worker) and identify specific duties at work that may pose a risk of transmission to others. Refer to [Table 1](#) for more information on SSO.
- If the case is a child, determine attendance at a childcare facility (e.g., daycare, dayhome) or other childcare arrangements, or school attended and grade.
- Determine the possible source of infection, taking into consideration the incubation period, reservoir, and mode of transmission. Assessment may include:
 - a history of recent travel or immigration, especially in areas with poor sanitation including improper water treatment and sewage disposal, either in Canada or abroad,
 - a detailed food history, especially consumption of contaminated ice/water, uncooked or undercooked food or food washed in contaminated water,
 - a history of risk behaviours including lifestyle risks for infection (e.g., MSM, IDU),
 - whether the case attends a childcare facility or other type of institutional setting (e.g., living in a correctional facility or residential/institutional setting),
 - whether there was any contact with a confirmed case of HA or contact with an ill person who had symptoms that were clinically compatible with HA infection,
 - a history of blood or blood product transfusion, or organ transplantation, and
 - similar symptoms in other members of the household (historical and present).
- Identify all contacts (including those in SSO that may pose a risk of transmission to others) that may have had exposure during the period that the case was infectious (period of communicability). Refer to [Table 1](#) for more information on SSO. Contacts include:
 - close personal contacts (e.g., household contacts, sexual contacts including MSM, regular babysitter/childcare provider, contacts in long-term care facilities),
 - persons who have spent 24 hours or more in the household,
 - persons who have eaten food prepared or handled by the case during the infectious period,
 - persons who have had or may have had indirect contact through sharing potentially contaminated items with the case (i.e., items handled by the case that could spread HAV through fecal-oral contamination),
 - persons in SSO, including staff,
 - persons who have shared illicit drugs with the case, and
 - others who may have had contact with the feces of the case (e.g., in the case of diapered children or others who are incontinent) where good standards of hygiene have not been met.

Table 1: Sensitive Situations or Occupations (SSO)

SSO	Definition
Food handler	- Touches unwrapped food to be consumed, <u>and/or</u> - Handles equipment or utensils that touch unwrapped food to be consumed.*
Healthcare, child care or other staff	- Has contact through serving food to highly susceptible persons. - Provides direct patient care and is involved in the care of young children, elderly or dependent persons.
Child attending a childcare facility or similar facilities	Is diapered or unable to implement good standards of personal hygiene.
Any individual (older child or adult)	Is unable to implement good standards of personal hygiene (e.g., with disabilities/challenges that may impact ability to perform good hand hygiene) and is involved in an activity that may promote disease transmission.

* NOTE: Generally, food handlers who do not touch food, equipment or utensils in this way are not considered to pose a transmission risk; however, circumstances for each case should be assessed on an individual basis.

Management of a Case

- All cases should be advised:
 - about appropriate personal hygiene, disease transmission, routine infection prevention and control practices, and contact precautions,
 - to avoid food preparation for others until symptoms have resolved, and
 - to avoid sexual practices that facilitate fecal-oral transmission.
- Contact precautions should be used in healthcare settings where children or adults have poor hygiene or incontinence that cannot be contained for at least one week after the onset of jaundice.⁽⁵⁾
- Advise the case to refrain from preparing food for others during the period of communicability.
- Notify and involve the Environmental Health Officer (EHO) when a food source is suspected.
- Refer to [Table 2](#) for case exclusion criteria.

Note: For clusters of locally acquired cases, consult with the ProvLab about submitting samples for genotyping.

Table 2: Case Exclusion

Cases	Category	Exclusion Criteria
Symptomatic	SSO	- The MOH may by order exclude a symptomatic case until diarrhea has resolved and for at least seven days after the onset of jaundice or at least 14 days after the initial onset of symptoms, whichever comes earlier. - The MOH may modify this exclusion period on a case-by-case basis (e.g., following the completion of appropriate post-exposure prophylaxis for contacts). - Currently available HA vaccines are 94–100% effective in preventing clinical disease.⁽⁵⁾
Asymptomatic	SSO	- Case-by-case review by the MOH. - May consult with the CMOH, as needed. Refer to Period of Communicability section for more information.
Either symptomatic or asymptomatic	Non-SSO	No exclusion required. However, all cases of gastroenteritis or enteritis should be regarded as potentially infectious and individuals should remain home from work, school or childcare while they are acutely ill.

Management of Contacts

- Assess all contacts, including visitors to the household, for potential of exposure during period of communicability for the case.
- Refer **symptomatic** contacts for STAT serology for anti-HAV IgG (immunity) and anti-HAV IgM. Asymptomatic contacts should generally not be tested unless SSO and do not have a history of immunity.
- Provide information about HA disease and measures that may be implemented to minimize fecal-oral transmission such as thorough handwashing:
 - after using the washroom and changing diapers, and
 - before eating and preparing or handling foods.
- Refer to Table 3 for contact exclusion criteria.

Table 3: Contact Exclusion

Cases	Category	Exclusion Criteria
Symptomatic	SSO	• Refer to their physician for assessment, if indicated. • The MOH may by order exclude a symptomatic contact until the contact has been assessed to rule out disease.
Asymptomatic	SSO	• Generally, no exclusion is required. - Contacts should monitor themselves for gastrointestinal symptoms, maintain good hand hygiene and food handling practices and seek medical attention if symptoms develop. If symptoms develop, the MOH may by order exclude the contact as per case exclusion. • Exclusion of asymptomatic contacts with no known immunity to HAV may be considered in special circumstances, such as food handlers who have had ongoing exposure to the case during the period of communicability and have not received PEP within 14 days of initial contact with the case. It is recommended to test for anti-HAV IgG (immunity) and exclude from work pending serology results. - If HAV IgG is negative provide PEP and lift exclusion. - If HAV IgG is positive, lift exclusion. - If HAV IgM is tested initially in error and is positive, request PCR testing to confirm result. IgM+/PCR+ indicates confirmed case, IgM+/PCR- indicates not a case. • If HAV symptoms occur at any time, request for HAV IgM to be added to existing testing and if positive, treat as a case.
Symptomatic	Non-SSO	• No exclusion. • Refer to their physician for assessment.
Asymptomatic	Non-SSO	• No exclusion. • Contacts should monitor themselves for gastrointestinal symptoms, maintain good hand hygiene and food handling practices and seek medical attention if symptoms develop.

Post-Exposure Prophylaxis

- Offer hepatitis A post-exposure prophylaxis (PEP) (0.1 mL/kg)⁽²⁹⁾ to all susceptible contacts as outlined in [Appendix 1: Hepatitis A Post-Exposure Prophylaxis Algorithm](#) and as detailed in [Appendix 2: Hepatitis A PEP Recommendations by Contact Type](#).
- PEP should be given ASAP and within 14 days of last exposure to the case (while the case was infectious).
- Hepatitis A vaccine may still be considered if more than 14 days have elapsed since last exposure, as there is no data on the outer limit of efficacy.⁽⁷⁾ This would be at the discretion of the MOH, on a case-by-case basis, and may be considered in high-risk situations.
- Refer to the current [Alberta Immunization Policy \(AIP\)](#) for vaccine and immune globulin (Ig) information.
- For setting-specific PEP recommendations refer to [Appendix 3: Hepatitis A PEP Recommendations by Setting](#).

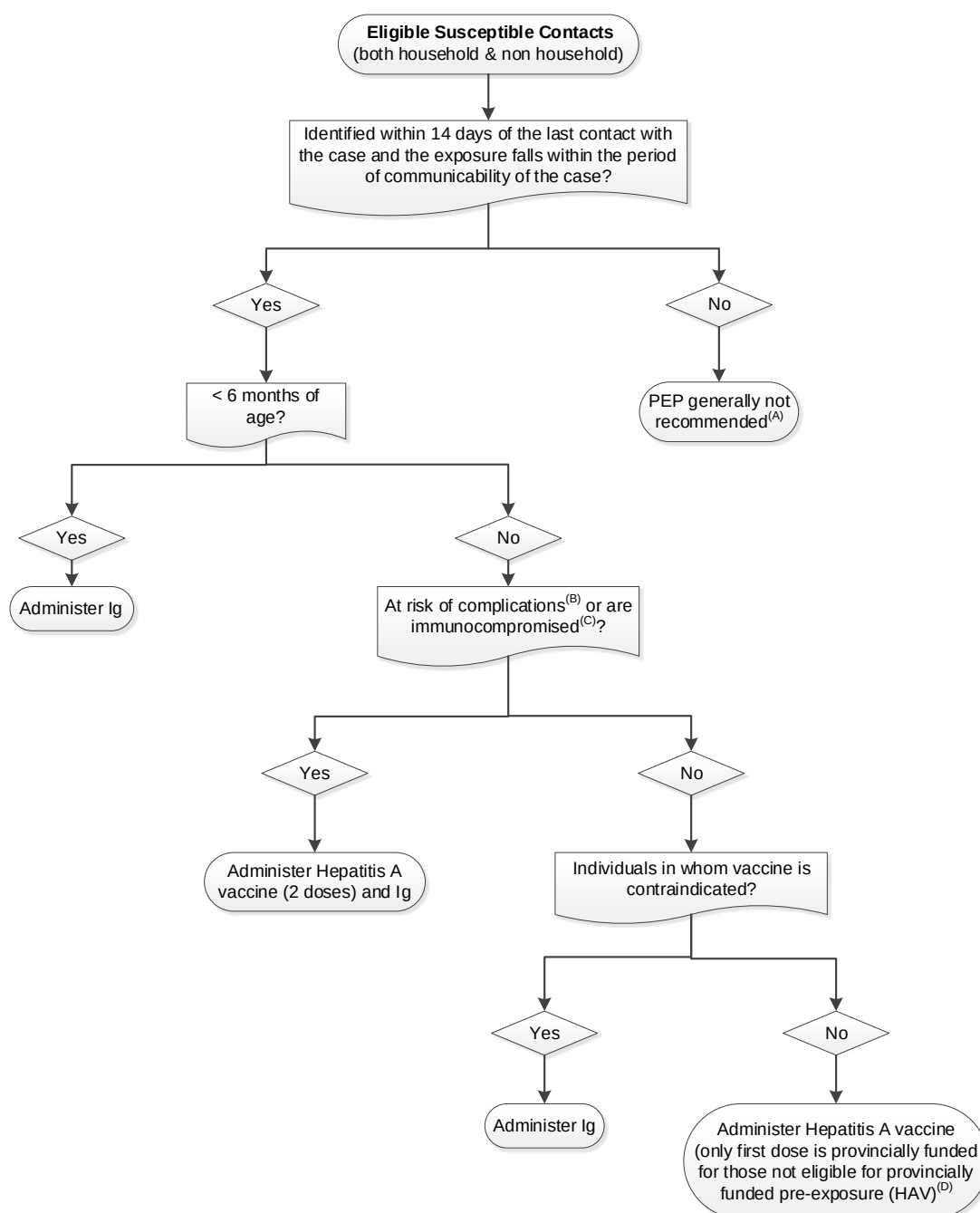
Susceptible Contact Definition

- No documented history of confirmed hepatitis A disease
- OR
- No documented record of the following:
 - Completed an appropriately spaced series of hepatitis A-containing vaccine (e.g., Havrix, Vaqta, Twinrix)
 - One dose of hepatitis A-containing vaccine between one and six months prior to exposure,
 - One dose of immune globulin (Ig) prior to exposure: time-frame is dependent on the dose received:^(8,10,20,29)
 - A dose of 0.1 mL/kg ≤ 1 month
 - A dose of 0.2 mL/kg ≤ 2 month

Preventive Measures

- Pre-exposure immunization is 90–97% effective in preventing HA infection.⁽²⁰⁾ Refer to the [AIP](#) for individuals eligible for provincially funded vaccine.
- Educate the public about the following:
 - personal hygiene, especially the sanitary disposal of items containing feces,
 - careful hand washing after defecation and sexual contact, and before/after preparing or eating food,
 - washing cutting boards, counter tops and utensils with soap and water after contact with uncooked foods, and
 - the risk of sexual practices that permit fecal-oral contact.
- Educate food handlers about:
 - thorough hand washing, and
 - proper food and equipment handling and hygiene, especially in avoiding cross-contamination from raw meat products.
- Encourage immunization for travellers to countries where hepatitis A is endemic.^(6,20)
- Advise travellers to developing countries about eating properly cooked foods and being cautious of uncooked produce and shellfish.⁽¹⁾

Appendix 1: Hepatitis A Post-Exposure Prophylaxis Algorithm



^(A) Hepatitis A vaccine may still be considered if more than 14 days have elapsed since last exposure, as there is no data on the outer limit of efficacy. This would be at the discretion of the MOH, on a case-by-case basis, and could be considered in high risk situations.

^(B) Individuals at risk of developing severe complications of hepatitis A: those with chronic liver disease; hepatitis B carriers; anti-HCV positive individuals; and those who have had or will be undergoing liver transplantation.

^(C) Vaccine efficacy may be reduced in the immunosuppressed, however, the vaccine will produce some protection and should be considered along with IG for post-exposure use when indicated.

^(D) Individuals eligible for provincially funded pre-exposure HAV – the opportunity should be taken to provide HAV to individuals eligible for pre-exposure HAV (Refer to the [AIP](#))

For those groups in whom pre-immunization serology for anti-HAV IgG is recommended, serology should be done prior to administration of Ig. However, Ig should be given as soon as possible and its administration should not be unnecessarily delayed.

Appendix 2: Hepatitis A PEP Recommendations by Contact Type

CONTACT	PEP (give ASAP)
- Those at risk of developing severe complications of hepatitis A: - those with chronic liver disease - hepatitis B carriers - anti-HCV positive individuals - those who have had or will be undergoing liver transplantation - Immunocompromised individuals^{(A)(20)}	HAV and Ig ^(B)
- Infants < six months of age - Individuals in whom vaccine is contraindicated (e.g., anaphylaxis to vaccine component)	Ig only
All other contacts ≥ six months of age not listed above	HAV only ^(B)

^(A) Vaccine efficacy may be reduced in an immunosuppressed individual however; the vaccine will provide some protection and should be considered along with Ig for post-exposure use when indicated.

^(B) If the individual is eligible for provincially funded pre-exposure hepatitis A vaccine as outlined in the current [AIP](#), BOTH doses of hepatitis A vaccine will be provincially funded. If the individual is NOT eligible for provincially funded pre-exposure hepatitis A vaccine as outlined in the current [AIP](#), only ONE dose of hepatitis A vaccine will be provincially funded for this exposure; however the individual should be encouraged to receive the second dose.

Appendix 3: Hepatitis A PEP Recommendations by Setting

SETTING	PEP RECOMMENDATION
Common Source Exposure and Infected Food Handler	• Circumstances should be evaluated on a case-by-case basis in consultation with MOH. • PEP should be offered (as outlined in Table 1) to other food handlers working at the same establishment as the infected food handler.^(5,30) • PEP is generally not indicated for patrons of the food establishment due to the rarity of common-source transmission. However, in consultation with the MOH, it may be considered if: – the case was likely to be infectious while working, AND – was involved in preparation/handling of unwrapped foods^A (either cooked or uncooked) and poor hygienic practices or had diarrhea, AND – contacts can be identified and provided with PEP within 14 days of the last exposure to the case during the period of communicability.^(5,30)
Childcare facilities (e.g., daycare, dayhome)	• Notify MOH (or designate) immediately if hepatitis A is reported in a childcare facility (in children or staff). • In childcare facilities that <u>provide care to diapered children</u>: – Offer PEP to <i>all</i> susceptible contacts at the childcare facility (staff and attendees). – If cases are recognized in two or more households of attendees, consider PEP for susceptible household members that have children in diapers attending the childcare facility.^(5,30) • In childcare facilities that <u>do not provide care to diapered children</u>: – Offer PEP to susceptible contacts (staff and attendees) <i>in the same room as the index case only</i>.^(5,30) • Keep the daycare operating and reassure parents that every effort is being taken to prevent further spread. • Discourage parents from enrolling their child in another daycare for the next seven weeks as a precaution against possible asymptomatic transmission. • Consider offering PEP to household contacts of diapered children who attend childcare facilities when outbreaks occur and cases have occurred in two or more families at the childcare facility.^(5,30)
Kindergarten	• Offer PEP as outlined in Table 1 to all susceptible contacts in the classroom (staff and attendees) of a case that occurs in kindergarten.⁽²⁰⁾
Schools and Work	• PEP is not routinely indicated when a single case occurs in a school or work setting.^(5,30) • Contacts with significant exposures, where there could be possible transmission of hepatitis A virus by the fecal-oral route, should be assessed and may be offered PEP on a case-by-case basis.
Health Care	• PEP is not routinely recommended in health care workers unless there is evidence of possible transmission of the virus by the fecal-oral route.^(5,6) • Appropriate infection prevention and control practices should be adhered to including strict hand hygiene.⁽¹⁾

^(A) Generally, food handlers who touch wrapped food, or food, equipment or utensils only prior to cooking, are not considered to pose a transmission risk however, circumstances for each case should be assessed on an individual basis.

Appendix 4: Revision History

Revision Date	Document Section	Description of Revision
December 2019	General	Updated guideline template.
	Case Definition	To acute clinical illness footnote – added that “Children may have non-specific or no symptoms”.
		Confirmed case – moved bullets around to provide better clarity from previous wording
		Probable case: - Added a footnote to clarify that “without laboratory confirmation of infection” means NO specimens were collected. - Removed “With a low positive anti-HAV IgM and PCR negative” as discussions with the lab indicate that low positive is equivalent to indeterminate and a PCR negative indicates this is a false positive.
	Epidemiology	Updated wording and references throughout
October 2021	Public Health Management	- Updated wording and references throughout - Added Sensitive Situations or Occupations (SSOs) table to inform the case and contact exclusion tables. - Removed “raw and without further cooking” as per discussions with Environmental Public Health (EPH) - Removed “to be consumed raw or without further cooking” as per EPH discussions. - Changed “child attending a childcare facility...” to “...school” as per <i>Public Health Act</i> s. 66(1)(d). - To “Any individual (older child or adult)...” added “...attending a public place” as per <i>Public Health Act</i> s. 66(1)(d). - Added a Case Exclusion table (converted from text bullets) - Added asymptomatic SSO exclusion to the case table. - Removed stat anti-HAV IgM after discussion with lab that testing asymptomatic contacts will result in false positives. - Added a Contact Exclusion table and what to do with test results for asymptomatic contacts. - Post-Exposure Prophylaxis (PEP): Annex 1: Hepatitis A PEP Algorithm notes reduced to decrease repetition. PEP recommendations by Contact Type moved to Annex 2. Hepatitis A PEP Recommendations by Setting moved to Annex 3.
	General	- Updated Template - Diagnosis and Treatment section moved to Epidemiology - Updated web links
	Reporting Requirements	Added subsections under AHS and FNIHB section to add more clarity as to reporting requirements by type of case (e.g. SSO, OOP/OOC).

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